



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : بدريه عبد المنعم ابراهيم عبد الحليم

العنوان : قرية العياط

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 26-09-2026

رقم البطاقة : 28103082100686 رقم التليفون : 01116424025

العمر (تاريخ الميلاد) : 08-03-1981 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____
التوقيع: (_____)

المقرر بما فيه : _____
التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة : _____
الوظيفة : _____
الوظيفة : _____

التوقيع : _____
التوقيع : _____
التوقيع : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / _____ أ.د. / _____ أ.د. / _____ د. / _____

قرار اللجنة

التشخيص النهائي : _____
خطة العلاج : _____
المستشفى : _____
جهة العلاج : _____
ملاحظات : _____

قسم : _____
نفقات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: بدره عبد المنعم ابراهيم عبد الحليم National ID: 28103082100686	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق المحمية

بدرية
عبدالمنعم ابراهيم عبدالحنيم
قرية العياط
مركز العياط - الجيزة

٢٨١٠٣٠٨٢١٠٠٦٨٦

KC3659160

٢٠٢٣/٠٨ ٢٨١٠٣٠٨٢١٠٠٦٨٦

بدون عمل

انثى مسلمة متزوجة
عماد محمد عبدالنواب سعداوى

البطاقة مارية حتى ٢٠٣٠/٠٨/١١

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 		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية		08/16/2026
MDT FORM				
Name: Badrya Abde Elminam Ibrahim Abd Alhalim		MRN: 2182		
Age: 45 Y		ID: 28103082100686		
Menopausal Sta: prem		Phone: 01124045633		
NRS SCORE:		Marital Status:		
BMI:		Family History:		
Gender:		Reason Of MDT:		
CO Morbidity:		met breast cancer to liver triple positive rec 32 weeks taxol herceptin with RD		
Surgical History: none		ECOG Performance Status ☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4		

Clinical Data:

BX:

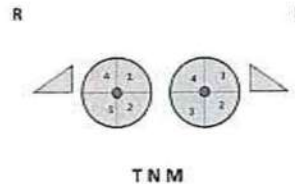
ER: 8/8 **PR:** 6/8 **HER2:** +3 **KI67:** 40%

History:

prem rt breast cancer met to liver ct4b n2 m1 met to liver cts bilobar hfl
 er8/8 pr6/8 her23+ ki 40%
 pt rec 34 weeks taxol and 11 cycles herceptin
 cts 2/6 rd of bilobar hfl largest seg 3 2.5x1.1 for mdt (to shift to ht)
 pt has grade 2 pn

Sonomammography:**Biomarker/Molecular****Mets Workup:****Recommendations:****MDT Outcome & Plan**

for femara zoladex and continue herceptin



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:**Pathological Staging:****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حنان عزت	د. احمد نجيب		

Name : Badriah Abdul-Moneim Ibrahim
Abdelhalim
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 18-09-2025 10:16

Medical No. : 205571

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 44-year-old lady, with right breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

^ 27/4
Patient's Name: Badriya Abdul-Moneim Ibrahim
Date: 2026-06-02
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

A known case of managed right breast cancer, as compared to the last CT examination dated 11/2/2026, the current examination revealed

I- CHEST:

- Still noted bilateral basal atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- The liver is enlarged size showing marked regressive course regarding size of bilobar variable sized non enhanced hepatic focal lesions, the largest is seen in left hepatic lobe segment III measures about 2.5x1.1 cm compared to 4.6x2.8 cm
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections. IUD is noted
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton no worrisome ytic or sclerotic lesions.

Opinion:

- Marked regressive course since the last CT examination as described.

Dr.Noirhan Ramzy

Patient Name : الأستاذ / بدييه عبد المنعم إبراهيم عبدالحليم	Request No. : 13002988
Patient No. : 28103082100686	Request Date : 18/08/2025
Path No. : SR-1129/25	Verification Date : 21/08/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (245-25). Together with, five referred slides including 1 H&E stained slide and 4 immunostained slides for (ER, PR, HER2neu & KI67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with focal mucinous differentiation.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * Minimal intermediate grade DCIS (Clinging pattern with comedo type necrosis & microcalcification).
- * Angioinvasion: Absent.
- * Perineural invasion: Present.

* IPT:

- ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 40%.

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 2, With minimal intermediate grade DCIS (Clinging pattern with comedo type necrosis & microcalcification).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).

Verified by :

Patient Name : الأستاذ / بديره عبد المنعم ابراهيم عبدالحليم	Request No. : 13002988
Patient No. : 28103082100686	Request Date : 18/08/2025
Path No. : SR-1129/25	Verification Date : 21/08/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- Her2: Positive (score 3+).
- Ki67%: 40%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Duaa Mohamed.

Verified by :

Patient ID: 86558	Patient Name English: Badriah Abd Almonem Ibrahim
Age: 44 سنة 4 شهر	2025/08/05 10:41:20 AM : Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 44 years complaining of right breast lump, no family history

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR density c).
- The right breast shows an UOQ segmental microcalcifications with an area of distortion noted at the axillary tail 8 cm from the nipple (the area of microcalcifications extends 8.2x6.5cm in dimensions).
- Prominent right Axillary LNs is noted.

Ultrasound Right breast:

- The right UOQ shows altered parenchymal echopattern extending from the retroareolar region upwards towards the axilla with foci of calcifications seen within.
- A hypoechoic spiculated lesion is noted at the axillary tail measures 20x18mm in dimensions with central vascularity seen 8.1mm from the skin another nearby small satellite 8x5.5mm.
- No cystic lesions.
- Pathologic level I axillary LNS (still noted fatty hilum) with diffuse cortical thickening the largest 15.5 mm in the longitudinal axis with cortical thickness 3.9mm and 15 mm in the longitudinal axis with cortical thickness 3.4mm.

Left breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR density c).
- No asymmetry, distortion or suspicious clusters of microcalcifications.

Ultrasound left breast:

- Preserved fibro glandular parenchyma.
- The left breast shows an oval hypoechoic probably benign lesion seen at 12 o'clock zone b measures 8x3mm in dimensions .
- No cystic lesions.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 5): Multicentric UOQ neoplastic breast lesion as described with DCIs component (the segmental microcalcifications as described) with pathological level I right axillary LNS.

Left breast (BIRADS 3): UOQ probably benign lesion as described.

Recommendation:

Right breast : for Tru cut core needle biopsy.

Left breast : 6 months interval follow up

Best regards,
Dr. Omneya.Ahmed.MD

Name : Badriah Abdul-Moneim Ibrahim

Date : 18-09-2025 10:16

Abdelhalim

Branch : Dokki

Medical No. : 205571

Ref.Physician Prof : Gostaf Roseh Hospital

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